

SECTION IV

Attachment and Relational Trauma

CHAPTER 12

The Stages and Styles of Attachment

HOW WE NORMALLY ATTACH

Gabor Maté and Gordon Neufeld state that attachment is at the heart of relationships and social functioning. They define attachment as the pursuit and preservation of proximity, closeness, and connection: physically, behaviorally, emotionally, and psychologically (Neufeld & Maté, 2014). Maté and Neufeld believe that there are six different ways children attach, which are through:

1. *Senses* (physical proximity)
2. *Sameness or identification* (being like the person you feel close to)
3. *Belonging and loyalty*
4. *Significance* (the feeling that we matter to someone)
5. *Feeling* (finding closeness through emotion, such as being warm, loving, affectionate, or emotionally intimate)
6. *Being known* (which they, like me, believe is the rarest of vulnerabilities and the hardest of all to do; Neufeld & Maté, 2014)

They also state that these ways of attaching underlie the drive for connection. This is at the heart of relational trauma: the natural desire to attach and connect, followed, unfortunately, by betrayal, abandonment, and violation.

Children do have a biological instinct to attach, which is a matter of survival (van der Kolk, 2014). Research on attachment suggests that the quality of parental care within the first two years of life promotes an “attachment style” for a child and sets a template for future relationships in adulthood (Fisher, 2017). Additionally, these early bonding experiences are later remembered not as visual or verbal narratives but as implicit memories and learned autonomic, motor, visceral, and behavioral responses. This sets the stage for affect tolerance, self-soothing, and an integrated sense of self later in life (Fisher, 2017).

It's generally believed that healthy parent-child interactions from a young age help children develop a secure attachment style that enables them to form stable and positive relationships in adulthood. However, when things go awry during this stage of development, it is also believed that lifelong relational difficulties can emerge.

ATTACHMENT STYLES

There are four main attachment styles that have been identified: secure, avoidant, anxious, and disorganized. Research shows that 62 percent of the general population is securely attached, 15 percent is avoidantly attached, 9 percent is anxiously or ambivalently attached, and 15 percent has what is called disorganized attachment (van der Kolk, 2014).

It is believed that children develop secure attachment when they grow up in an environment where their mother is responsive and attuned to their needs. *Securely attached* children develop an internal locus of control. They know what makes them feel good or bad, and they have developed a sense of agency (van der Kolk, 2014).

However, children who are raised by a mother who is unresponsive or rejecting become *insecurely attached*, causing them to respond in one of two different ways. (Consider, however, that all the prominent research published involves studies of mothers, which I have issues with for obvious reasons. After all, what about the role of fathers, especially when they are the primary caregiver?) One reaction of the insecurely attached child is to become *anxious* or hyperaroused, often crying, yelling, complaining, and acting needy. The insecurely attached child can also become *avoidant* or hypoaroused. These children act aloof and uninterested, as if nothing matters or bothers them (van der Kolk, 2014).

Meanwhile, children develop *disorganized attachment* in response to mothers who appear to be hostile or intrusive and helpless or fearful (Lyons-Ruth & Block, 1996). These children appear frightened without having a clear solution for connection. Karlen Lyons-Ruth, a professor of psychology at Harvard University, has studied attachment and become particularly interested in this disorganized attachment group. She has found that a mother's emotional withdrawal, not her intrusiveness, has the most profound and long-lasting effect on her child's mental instability (Lyons-Ruth & Block, 1996). Lyons-Ruth has also found that children are much more likely to develop dissociative symptoms in young adulthood if they are raised by mothers who are disengaged and misattuned during the child's first two years of life. These young adults feel lost, overwhelmed, and disconnected and see themselves as unloved, empty, and helpless (Lyons-Ruth, 2003).

Attachment theory demonstrates the importance of healthy “good enough” parenting on a child’s development and also alerts us to the mental health and relational difficulties that can ensue when a child is relationally neglected, ignored, and repeatedly intruded upon. This data is consistent with the ACE study results discussed in [chapter 2](#).

A DIFFERENT VIEW OF ATTACHMENT DISORDERS

I agree with the necessity and importance of good quality connection for the safety and well-being of children. Children cannot survive without being attached. Connection is one of the most important things in life—for both children and adults.

However, I have a different view of attachment styles as they relate to IFS therapy. I don’t necessarily believe in attachment styles per se, nor do I believe that they get firmly established within the first two years of life. *I do believe that different parts of children attach to different parts of caregivers throughout their life.* For example, a part of me developed an anxious attachment to the part of my mother that often denied reality. Another part of me developed an avoidant connection to the part of my mother that could be intrusive at times.

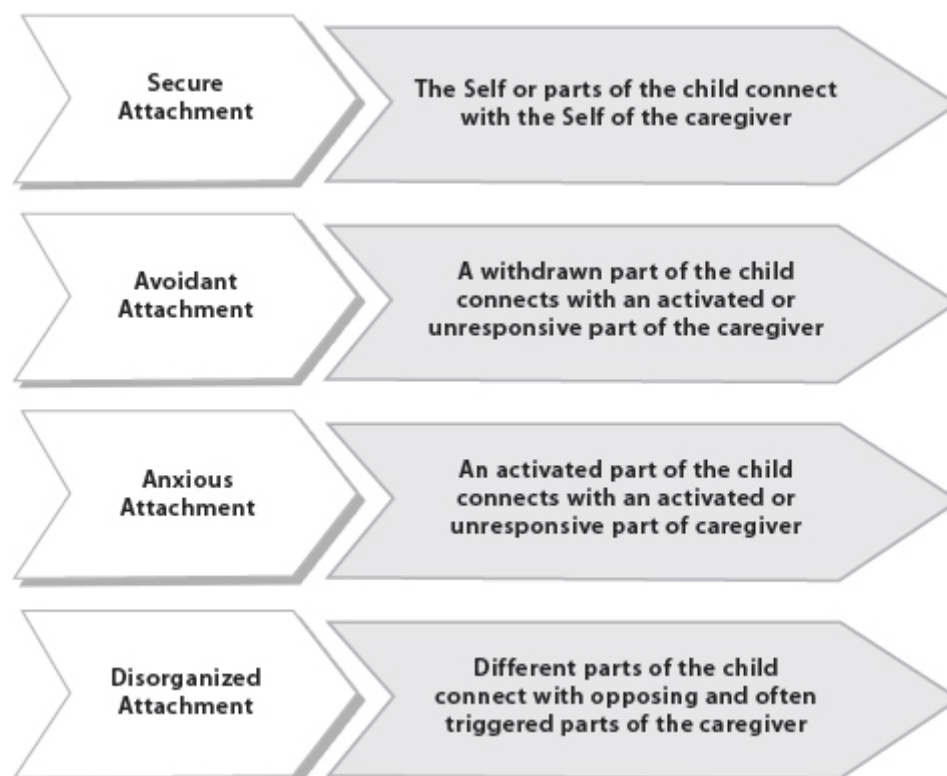
I also believe that parts of me securely attached to the parts of her that were eternally loving and supportive (i.e., her Self). I have parts that developed a disorganized relationship to her confusing and often contradictory parts too. I don’t think these are permanent or entrenched styles of me, and I don’t believe this is the way I relate to most of the people in my adult life. *I contend that most attachment styles, when seen through an IFS lens, are wounds or protective parts that develop as a result of difficult or challenging interactions* and that they have a big influence on our lives as adults, especially when they’re not adequately addressed or healed. I contend that we each have different parts that relate to different parts of other people. I further posit that we have experiences with each of these “styles” or “different parts,” which connect to the various parts of people with whom we’re in connection.

I have experienced this same phenomenon from the opposite side—as a parent with my children. They know whom to ask when they want someone to cook for them, whom to approach when they want to go biking or play on the trampoline, and whom to avoid when they fight with each other. I believe that both of my boys feel safe and secure when they are in connection with me when I’m in Self-energy. I also observe that they’ve had confusing and disorganizing experiences when I lose my temper and become someone totally different to them in those moments. At first, they connect with the reasonable, calm, non-triggered part of their father; it undoubtedly shocks them when an extreme, irrational, and intense part of me takes over. One minute I’m the reasonable father they know and

love, and the next moment I'm triggered and present in a completely different way. This must be confusing for them, especially if I don't take responsibility for my actions and fail to apologize after I've recovered. Unfortunately, this is the case for many complex trauma survivors: The parent doesn't own their piece, and the child internalizes it, assumes responsibility, and tries to make sense of it on their own.

I often encounter clients who would certainly fit a disorganized attachment style. But when you approach them using an IFS lens, you quickly see that there are two different parts with two different views that have developed as a result of complicated relational dynamics. These clients often have parts that think: "Relationships are dangerous; I must stop them at all costs" (a protector) and "I'm desperate for connection; I'll do whatever it takes to keep that person in my life" (a wound). I feel that disorganized attachment can be more disorganizing for the person who is in relationship with these two different parts, not for the person who holds the opposing views. But when we see this from an IFS perspective, we take a curious stance and listen to both parts; they both make perfect sense. It becomes clear, not disorganized.

Here is how attachment theory can be viewed through an IFS lens:



CHAPTER 13

IFS as Internal Attachment Work

THE POWER OF ATTACHMENT TRAUMA

The parts that develop as a result of ongoing young attachment wounding can be potent and subtly pervasive. They can dominate a person's life, especially when that person enters adulthood and attempts to engage in intimate relationships. I believe that most, if not all, attachment wounds live within implicit memory, which is persistent, largely unconscious, and has no time sequence or narrative associated with it.

I've encountered many clients who struggle with this issue. In thinking about this problematic prevalence, the song "Run the World (Girls)" by Beyoncé comes to mind, funnily enough. Only instead of the standard lyrics, "Who run the world? Girls," in my mind the lyrics change to "Who run the world? Exiles." Because exiles, the parts that carry wounds, are legion. Think about it: Young and desperate attachment wounds actually *do* run the world and make most of our relational decisions for us. *Attachment trauma couples the past with an attempt at redemption in the present.* IFS helps clients to "uncouple" the past from the present, making the pull to relive much less likely.

Adults typically have free will and choice when it comes to decisions about relationships. They can choose to stay, or they can choose to leave a painful, unsatisfying, or abusive relationship. Children are not afforded this same luxury; they are forced to stay. When adults suffer from a history of repeated relational betrayals when they are young, and they haven't done the work to heal those wounds, they tend to repeat them. Adults often remain tethered to their past, believing they can't leave, and reenact their childhood experiences with their adult partners.

These young attachment wounds make most of the relational decisions in adulthood, and they do so in the service of trying to fix the past. They may think, for instance: "This guy will treat me better than my father did" or "She'll love and accept me in a way that my mother never could." But these parts still hold past beliefs and internally respond with thoughts like: "I can't leave. Men have all the power—they're always in charge" or "Women are usually controlling. If I speak up, I'll most likely get criticized." These exiled parts are at the root of attraction and desire, but they're ill-equipped to maneuver adult relationships in a healthy, mature way. This sets the stage for repeated, abusive relational reenactments of young attachment wounds in adulthood.

Healing these wounds allows one to relationally choose from a different place, not from a place of the past or out of hope for repair and redemption but from a place of adult openheartedness and Self-energy. When working with clients who haven't dealt with their attachment trauma, we often feel the energy and power of these young parts. Yet they're often below the radar and rarely named. However, these young parts are ever-present and palpable and have a powerful influence over our clients' lives. And when we finally acknowledge and address them in therapy, these exiles can take months to years to heal.

DIFFERENT ATTACHMENT WOUNDS

In my experience, these attachment wounds often consist of a series of hurts and betrayals that occur over several years and are held in one main part. I call this central part the *collective attachment wound*, which is a series of traumas brought together under one main exile. This exile is often referred to as “my little one” or given a name like “Little Tony” or “Tiny Samantha.”

Consider the plight of a client we'll call Daniel. Daniel was a high-achieving computer engineer, always at the top of his class. He constantly attempted to date the most attractive woman he could find to subsequently marry and have children with. After many failed attempts, Daniel finally found what he called the “perfect wife,” with whom he raised a family. Daniel would frequently come into therapy sessions following a disagreement with his wife, saying: “I'm such a failure, I can't stop her from being mad at me. I wouldn't blame her if she decided to leave.”

Nothing was ever good enough for Daniel, and he was never satisfied with how things turned out in his life. But after meeting many of his intense protective parts, Daniel was able to connect with a younger part of him who grew up feeling “less than” and inadequate. Daniel's life suddenly had a context that made sense to him.

“I'm blown away right now meeting this ‘little guy,’” he told me. “I can't believe how every single choice in my life up to this point has been driven by him. He feels so insufficient and defective. Most, if not all, of my decisions have been a desperate attempt to try and counteract how this little guy feels about himself. This work we've done on my parts is incredible.”

It's also possible in attachment trauma to have several different exiles hold similar painful experiences over many years. I refer to these as *developmental wounds*, which we will discuss further in [chapter 23](#). Case in point: A 3-year-old part, a 9-year-old part, and a 16-year-old part all carry the experience of being physically abused by their mother. This type of wounding can also have a powerful impact on adult relational decision making.

Consider the case of Olivia, a pediatrician who came in to see me because she was unhappy in her second marriage. She had three children: two from a previous marriage and one with her current partner. She struggled with a harsh internal critic part that would also intermittently lash out at her husband, as well as an intellectual part that kept her away from most, if not all, of her feelings. But her clinical profile never really made sense to me. She didn't seem to have a history that was consistent with her symptoms and life struggles. I couldn't connect her intimacy difficulties with any comprehensible history.

Slowly, we were able to perform cumulative healing work that was consistent with what many therapists would call "small 't' trauma." We helped a part Olivia had that felt ignored as a child while away at camp. Another part that felt less important than and in the shadow of her older sister. A third part that was fiercely yelled at by her father, once when he was "overserved" at a wedding. And a different part that felt shamed in middle school by the popular girls who teased her for not being athletic. Much to our surprise, Olivia began to feel much stronger and more confident in her life. She was able to speak up at work and at home with her husband in fresh, positive, and constructive ways. She became a stronger parent and felt increasingly happier, now desiring to taper off her prescribed antidepressant.

"I had no idea that all those events in my childhood had such a huge impact on me," she said. "I realize that each of these experiences held some shame and the feeling of being less than. I think all the little pain added up over time and had a huge impact on me."

"I'm so happy that we were able to help all your parts release what they were holding onto," I replied.

It's important to keep in mind that not all of the energy and decisions motivated by attachment wounds are negative. These young parts can create stamina and an internal drive that can help clients achieve some incredible things in their lives. "I would never have gotten through medical school had it not been for those younger parts of me," Olivia said. "These parts have helped me so much throughout my life, now it's time for me to help them."

SEPARATION AND PREVERBAL TRAUMA

With attachment wounds in particular, separation—or helping parts unblended from the Self—can be challenging. These parts often present as if they are holding on for dear life, and the need to stay connected often feels to them like a matter of survival. The idea of softening or detaching from the Self, even slightly, seems utterly impossible.

Here's where therapist creativity comes in handy. I suggest finding imaginative ways to convince your client's young needy parts that separation is possible and helpful, despite this supposed paradox of separation. For example, I'll often engage in direct access with these young parts, saying something like: "If you're willing to separate from Dan just a little bit, he'll be able to be with you in the way you've always wished and you won't have to be alone any longer." I'll then make a suggestion that includes some form of separation accompanied by continued connection: "Maybe if you hold his hand or sit on his lap and try looking at Dan, you'll be able to feel more connected to him?" Your goal is to be imaginative and resourceful with suggestions that encourage separation yet still allow for contact, which these parts crave and desperately need to feel safe.

Attachment trauma can also show up in preverbal forms. Remember that within the first two years of life—when only implicit memory is present—abandonment, betrayal, and intrusive experiences are encoded without words. Parts are, however, still quite capable of communicating, even when language is not developed. They can express themselves through feelings, physical sensations, visual images, sound, and amazingly, transmission of thought. I encourage my clients to be open to all of the ways their parts want or need to communicate with them.

There tends to be an order of communication when it comes to preverbal parts. It typically follows *a bottom-up pattern of transmission*. For example: Physical sensations present themselves first, followed by the expression of emotion, and last, the story or narrative reveals itself. It's also common for preverbal parts to *start out inanimate or one-dimensional and to evolve into human form* as the Self gets to know them better.

Take the example of Fatima, a client of mine who would often say that her body felt the need to move as she sheepishly asked me if she could lay down on the couch in my office during sessions. She slowly began to move in an unorthodox rhythmic manner for several sessions in a row before she also began to cry. I had the sense that I just needed to stay present and to hold space for her as this process unfolded, not really knowing where it was heading. Eventually, the sensations and feelings were connected to a memory that she was ultimately able to put into words. She described a baby in the back seat of a car who witnessed her brother tragically die in a car crash. This preverbal memory, of which Fatima had no prior awareness, was eventually unburdened, and much to her surprise, her lifelong fear of driving also resolved.

We can even encounter parts who communicate in utero, as demonstrated by the following case example. Here, it's important to have any of your skeptical parts step back and to allow the process to unfold organically. These scenarios can also be difficult for clients to trust, but, over time, they usually make sense and fit together like pieces of a puzzle in the context of their life.

BEFORE WORDS—BRANDON (TO BRIANNA)

Brandon, a 24-year-old who is in the process of transitioning, recently encountered a roadblock in therapy:

“For some reason, I’m really struggling with using she/her pronouns. I know I’ve always felt trapped in this male body, and my Self knows I’m a woman, but something is preventing me from taking this next step in my transition.”

“Well, let’s check inside for a moment and see if we can learn more,” I said.

“I just hear, ‘We’re wrong, we’re wrong, no one will ever love us.’”

“Let those parts know that you’re hearing them, Brandon, and that we’re here to help them.”

“Now I’m now seeing the parts that tried so hard to fit in. I see myself looking at Uncle Mike, trying to act and move like he did so I would look normal. I see myself playing Star Wars with the boys next door, always secretly wanting to be Princess Leia but knowing to keep those thoughts to myself. I even remember the look of disgust I got from people when one of my girl desires accidentally leaked out. I was trying to be someone else my whole entire childhood. It’s so sad.”

“Yes, it is, Brandon. Try to send these parts some love and appreciation for the ways they tried to help you over the years.”

“They really like that. They really saved my life, you know.”

“Yes, I know they did,” I responded.

“I’m now seeing a tiny infant in what appears to be a womb or something. It sensed that something was drastically wrong. I’m getting the sense that the baby felt defective and unworthy of love,” Brandon said.

“Brandon, if it feels right in your heart, can you let the infant know you’re getting all of this? Can you send it love and let it know it’s perfect just the way it is?”

“It really loves hearing that and, for the first time ever, is able to take in love. It’s relaxing and seems so calm right now. This is exactly what this baby needed. It’s incredible, it’s letting me know from its heart that we were a ‘her’ from the very beginning and that our name is Brianna, not Brandon.”

“That’s beautiful,” I responded with a tear in my eye. “Let the infant know that we’re going to do our best to help the outside better match the inside.”

“She loves that!” Brianna said.

THE CLIENT’S SELF AS THERAPEUTIC AGENT: REPAIRING THE INTERNAL RELATIONSHIP

Attachment-centered therapies tend to focus on developing or rebuilding trusting relationships between the therapist and client, with the hopes of correcting and repairing the client's unhealthy attachment patterns that developed earlier in life. I believe that attachment and connection are more than merely important: They're vital in healing the chasm created by complex relational violations. However, as I've stated previously, IFS maintains that: (1) the relationship between a client's Self and their wounded part is primary, and (2) the therapeutic relationship between the therapist and client is secondary regarding healing wounds. No one can correct or repair the hurt caused by the past like the Self can—not even the best of therapists.

ATTACHMENT AND THE CORRECTIVE EXPERIENCE

During a therapy session, a client of mine named Joan achieved an unburdening with a younger part of her that felt left out and ignored by her summer camp counselors when she was 11 years old. A week later, Joan arrived for her weekly session.

Here is the dialogue we engaged in:

“How is your younger part doing? Have you been checking in with that 11-year-old daily as you promised you would?” I asked.

“Things are going pretty well, I think. The girl is starting to feel a new level of confidence within herself,” she said.

“That's great, Joan. But what do you mean by ‘I think?’”

“While I was checking in with her over the week, she moved back into the past and I'm not sure why,” Joan said.

“Is there more she wanted to share?” I asked.

Joan's 11-year-old part proceeded to tell Joan about her older sister Samantha—and how much the young Joan loved, looked up to, and missed her elder sibling.

“I wanted to be just like her. She was interesting, and I wasn't. Everyone liked Samantha,” the young part replied.

“Let's ask your younger part more about that, Joan,” I said.

After a few moments of silence, the woman before me quietly began to cry.

“It was all about my mother's love and attention,” Joan said. “That little girl knew my mother loved her oldest daughter more. Samantha was smarter, prettier, and more well-liked by all the children at camp. The little girl thought that if she was more like Samantha, maybe her mom would like her too.”

We both paused for a moment, reflecting on the feelings behind the statement. I asked, “Joan, what would you like to say to her about that?”

“I’m brokenhearted that you didn’t get the attention you needed from Mom,” Joan’s Self told her 11-year-old part. “You are interesting. Your day was interesting. You are worth listening to. You are lovable. I love you.”

“Is she listening to you and receiving what you’re saying?” I asked.

“Yes, totally. I love that I can time travel and give her what she needed and wanted back then. She is so happy right now. We are walking together and holding hands.”

“That’s beautiful,” I told her with a smile.

“What would you like to say to her about that?” is one of those very important questions we ask when Self-energy is present with an exiled part. *The Self is the ultimate corrective experience*. When the original person—in this case, Joan’s mother—isn’t able to offer love, comfort, and healing, the Self serves as the next best thing. Here, the Self is incredibly powerful and giving—and in speaking from the heart, it provides the part what it needs and wants.

Beware: Parts easily get attached to the therapist as the hoped-for corrective object. Nine times out of ten, the part will choose the therapist over the client’s Self as its healing agent. Remember that the therapist didn’t abandon the part. That’s why it’s important to be mindful of our therapist caretaking parts here. If we’re not careful, we can foster this relationship in a way that reinforces the breach between the client’s Self and the part. The more attached these parts get to us, the harder it becomes, as Dick Schwartz says, to *pass the baton* back to the client’s Self as the primary healing agent, which is the goal.

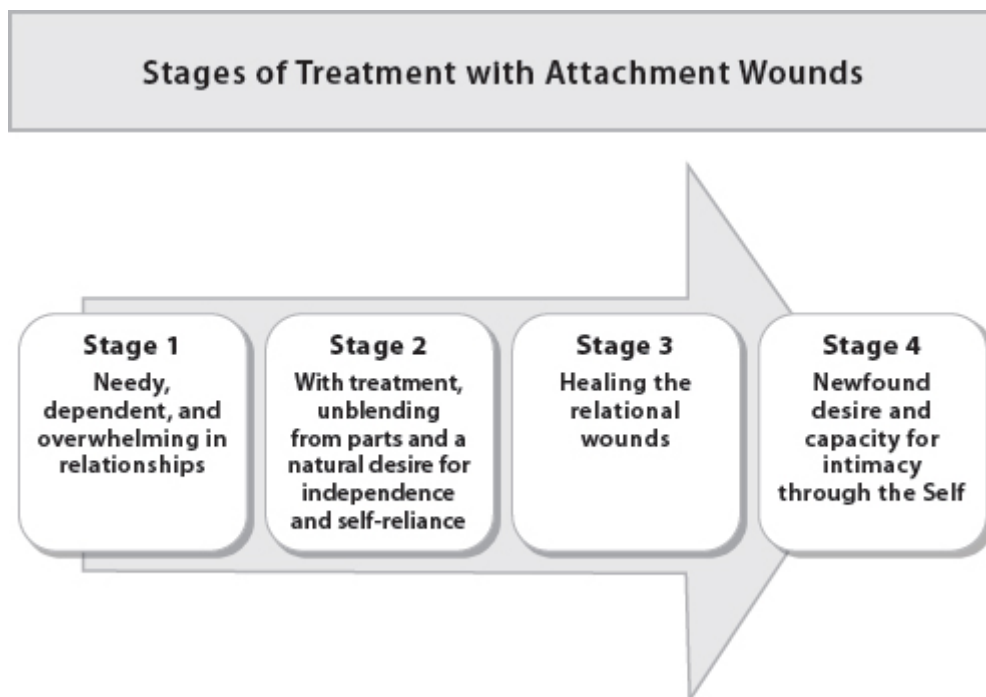
RELATIONSHIPS, TREATMENT, AND ATTACHMENT TRAUMA

During treatment with survivors of attachment trauma, there’s often a particular order or rhythm that occurs when it comes to their adult relationships. Initially, many survivors will be overly dependent on the people they’re connected with. Their younger parts show up and can be experienced as desperate, needy, and overwhelming to those around them. When clients enter treatment and therapists begin the process of helping them begin to separate from their younger parts, a natural shift occurs in the needs and priorities of their external relationship. During this stage, clients tend to become more independent and self-reliant. They often end unhealthy, codependent relationships and spend time getting comfortable with themselves, sometimes preferring to live alone for a while.

I often get excited for my clients during this stage of beginning independence, which is important for them both internally and externally. I may think to myself: “Finally, Jim can stand on his own two feet, be on his own, and disengage from unhealthy serial relationships. Jim is getting comfortable with himself and his parts, and for the first time in his life, he doesn’t need someone else in order to function and be happy. He is benefitting from the strength of self-independence.”

As the therapist, it’s important to observe this stage carefully and to ensure that the client doesn’t get *too* comfortable and settle in for the long haul. Using the previous client as an example, consider that Jim could become happy on his own and start to avoid relationships and intimacy altogether, as his memories of them are painful, heartbreaking, and unsuccessful. Instead, I want to help Jim appreciate his newfound separateness but not let him get too entrenched in this comfort zone. I need to gently nudge Jim into gravitating toward past pain so that he can begin healing deeply buried relational wounds. Then I want to guide him toward a place of open curiosity and nurture a true desire for connection with someone from a place of Self-energy. I don’t want him to remain in a place of isolation, avoidance, and fear of intimacy.

In summary, clients with attachment trauma often start out needy in relationships and can settle into an unhealthy zone of solo comfort. It’s our job as therapists to assist them in addressing their pain, healing their wounds, and emerging with a fresh desire for intimacy that’s genuinely driven by their Self, not by their redemption-influenced past parts.



NEUROSCIENCE: THE SCIENCE BEHIND CONNECTION

Lane Beckes and James Coan, researchers from the University of Virginia, have conducted research demonstrating the obvious: *connection is important and valuable*. However, what is unique about their work is that they have validated their findings from a neuroscience perspective. Social baseline theory (SBT), as they call it, holds that the presence of other people helps us conserve important and often metabolically costly somatic (bodily) and neurological resources through the social regulation of our emotions (Beckes & Coan, 2011). In other words, relationships help us manage our emotions without needing to utilize our own physical or neurological resources.

Beckes and Coan have determined that the quality of our connections helps us regulate or recover from negative emotions in the brain. The neural structures we typically recruit to manage our emotions are *less* active when high-quality social support is available. When we maintain social connection, our brains are less vigilant in response to threats, and we do not need to engage in emotional self-regulation. Put another way, closeness represents an innate *baseline* strategy for human emotion regulation, one that doesn't require deliberate action on our behalf (Beckes & Coan, 2011).

These researchers have also studied how women respond when confronted with a shock or threat across three different scenarios: while they were alone, holding a stranger's hand, or holding a partner's hand. Women with the highest-quality relationships (i.e., holding a partner's hand) required the least brain activity to recover. The physical connection with their partners allowed them to recuperate more quickly while expending less brain energy. As the quality of this connection lessened, they needed to recruit more brain structures to recover. All three groups of women recruited the right anterior insula, superior frontal gyrus, and hypothalamus. However, women who held a stranger's hand *additionally* recruited the superior colliculus, right dorsolateral PFC, caudate, and nucleus accumbens. Women who were alone also engaged the ventral anterior cingulate cortex, posterior cingulate, supramarginal gyrus, and postcentral gyrus to recover from the threat.

The point here is not to memorize all of the brain structures involved in rebounding from a threat. Instead, it's useful to appreciate the importance of connection as it relates to recovering from stress and trauma. *Self-regulation*, or recovering from a threat while alone, requires our brain and body to work hard, utilizing excess energy and internal resources to help inhibit or downregulate the emotional activation caused by a peril. With a safe, high-quality connection present, the women didn't need to expend energy to bounce back from the threat; they didn't even mount an emotional response.

Having a connection was shown to decrease cardiovascular arousal, facilitate the development of a non-anxious temperament, inhibit the release of stress hormones, reduce threat-related neural

activation, and generally promote health and longevity. Conversely, social subordination, rejection, and isolation were shown to be powerful sources of stress and compromised health (Beckes & Coan, 2011).

In summary, attachment and connection are important to all of us at any age. When we're engaged in healthy, higher-quality relationships, our parts are less often activated, the Self is more present, and it's easier and less taxing to overcome a violation. When we're alone or in unsatisfying relationships, we can still recover but our protective parts get triggered more easily, we expend more emotional energy, and we stress our nervous system in response to a perceived danger.

Under more severe circumstances or with repeated relational trauma, it becomes much harder, if not impossible, for us to recuperate. Extreme parts, ranging from hyperaroused to dissociative, become activated and attempt to fend off the pain and anguish caused by the assault. When recovery isn't possible, PTSD develops, symptoms persist, and parts get stuck in time, either trying their best to keep overwhelming feelings locked away or constantly reliving these emotions each time they get triggered.

Fortunately, IFS offers hope and a way to reverse and release the chronicity of the relationally traumatic experiences for all those who have been afflicted.